

Final 41 OBS

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ
{رَبَّنَا أَخْرِجْنَا مِنْ هَذِهِ الْقَرْيَةِ الظَّالِمِ أَهْلُهَا وَاجْعَلْ لَنَا مِنْ لَدُنْكَ وَلِيًّا وَاجْعَلْ لَنَا مِنْ لَدُنْكَ نَصِيرًا}

I) Short essay questions (15 marks)

Q1- Define preterm Labour (1 mark), enumerate 2 causes (1 mark) and list lines of treatment (3 mark)

Q2- Define internal podalic version (1 mark), mention its indications (1 mark) and list precautions and technique (3 marks)

Q3- Define habitual abortion (1 mark), enumerate 2 causes (1 mark), 2 investigations (1 mark) and mention lines of treatment (2 marks)

II) Problem solving questions (20 marks, 5 marks for each question)

Q4- 30 years old patient, known to be pregnant 10 weeks, came to emergency department complaining of vaginal bleeding, her B subunit was 121000 mIU/ml, trans-vaginal ultrasound was done and revealed snow storm appearance

1-What is the incidence of this condition? (1 mark)

2-Enumerate 2 risk factors of this condition? (1 mark)

3- Outlines treatment of this condition? (2 marks)

4- Mention post treatment follow up of this condition? (1 mark)

Q5- 26 years old patient, came to emergency department complaining of decreased fetal movement, her last menstrual period was at 6/10/2024

- 1-What is the gestational age according to LMP? (1 mark)
- 2 - List 2 causes of this condition (1 mark)
- 3- How to diagnose this condition? (2 mark)
- 4- List 2 complications of this condition 1 mark)

Q6- 23 years old patient, G6P5+1 pregnant 37 weeks, cephalic presentation. She came to emergency room complaining of labour pain which started 1hr before. By examination, Bp. pulse and temperature are normal. Her U/S showed that fetus is living and cephalic presentation. By PV examination, the cervix is fully dilated and the mentum is the presenting part

- 1- Enumerate 2 causes of this condition? (1 mark)
- 2- Describe mechanism of labour of this condition? (2 marks)
- 3- List lines of management of this condition? (2 marks)

Q7- Ms. Nadia 29 years old, G4P3+0 pregnant 39 weeks, cephalic presentation. She came to labour room. By PV examination, the cervix is fully dilated and she delivered the fetus immediately. When the doctor try to deliver the placenta, he tries many times to pull the cord down while the uterus is still not contracted, and try to do vigorous fundal pressure suddenly the fundus collapsed into uterine cavity

- 1- What is the etiology of this condition? (1.5 marks)
- 2- Enumerate degrees of this condition (1.5 marks)
- 3- Mention lines of treatment of this condition (2 marks)

Q8. Extended matching questions (5 marks, 1 mark for each question)

1- A 40-year-old pregnant woman at 16 weeks' gestation with a BP of 144/95, no edema, no proteinuria (.....)	A) Mild preeclampsia
2- A 19-year-old pregnant woman at 36 weeks' gestation with a BP of 150/100, and 2+ proteinuria with no other symptoms (.....)	B) Severe preeclampsia
3- A 21-year-old pregnant woman in early labor at 39 weeks' gestation who has just convulsed (.....)	C) Chronic hypertensive disease
4- A 23-year-old pregnant woman at 37 weeks' gestation with a BP of 145/105, 2+ proteinuria, and elevated liver enzymes (.....)	D) Eclampsia
5- A 35-year-old pregnant woman at 32 weeks' gestation with a BP of 180/120, no proteinuria or edema (.....)	E) Chronic renal disease
	F) Gestational hypertension
	G) Superimposed preeclampsia

Q9- Multiple choice questions (10 marks, 0.5 mark for each question)

1- The following are predisposing factors for placenta previa, except

- A) Advancing maternal age
- B) High older pregnancy
- C) Previous caesarean section
- D) Malpresentations
- E) D&C

2- The following statements are related to cord prolapse except

- A) Common in malpresentation
- B) A loop of cord may be brought down to confirm the pulsation
- C) One of the first aid management is to lift the presenting part of the cord
- D) High risk of fetal distress
- E) Treated by emergency CS

3- A 39 year-old pregnant lady with gestational age of 38 weeks. CTG monitoring revealed transient decrease in fetal heart rate after beginning of uterine contractions and subsided after ending of the contractions. What is your diagnosis

- A) Early deceleration
- B) Late deceleration
- C) Variable deceleration
- D) Sustained deceleration
- E) Bradycardia

4-The oxytocin infusion is contraindicated in all except

- A) Obstructed labour
- B) Trial of labour
- C) Hypovolaemic shock
- D) Cardiac disease
- E) Cephalo-pelvic disproportion

5- A 25 year-old pregnant lady at 39weeks of gestation. During labor, a hand is found beside the head in cephalic presentation.

What is your diagnosis

- A) Occipito-anterior position
- B) occipito-posterior position
- C) Face Presentation
- D) Brow presentation
- E) Compound presentation

6- What is the station when the fetal head is of the level of chin spines

- A) -2
- B) +2
- C) +1
- D).0
- E) -1

7- Vaginal examination during pregnancy is contraindicated in

- A) Carcinoma of the cervix
- B) Gonorrhea
- C) Prolapsed cord
- D) Placenta previa
- E) Active labor with ruptured membranes >6 hours

8- All of the following are components of biophysical profile except

- A) Oxytocin challenge test
- B) Non stress test
- C) Amniotic fluid volume
- D) Fetal respiratory movement
- E) Fetal body movement

9- The most constant symptom present in undisturbed ectopic pregnancy is

- A) Lower abdominal pain
- B) Vaginal bleeding
- C) Amenorrhea
- D) Shoulder pain
- E) Shock

10- A woman with 20 weeks gestation presents with vaginal bleeding. On speculum examination, the cervical os is open but no products have come out. The diagnosis is

- A) Inevitable abortion
- B) Missed abortion
- C) Complete abortion
- D) Incomplete abortion
- E) Threatened abortion

11- With reference to fetal heart rates considered reactive when

- A) Two fetal heart rate accelerations are noted in 20 minutes
- B) One fetal heart rate acceleration is noted in 20 minutes
- C) Two fetal heart rate accelerations are noted in 10 minutes
- D) Three heart rate accelerations are noted in 30 minutes
- E) Fetal heart accelerations during uterine contractions

12- A 28 years old female with history of 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound examination there is gestational sac with absent fetal pulsation. The diagnosis is

- A) Inevitable abortion
- B) Missed abortion
- C) Threatened abortion
- D) Incomplete abortion
- E) Vesicular mole

13- Ligation of which artery can control postpartum hemorrhage

- A) Origin of internal iliac artery
- B) Anterior division of internal iliac artery
- C) Posterior division of internal iliac artery
- D) Common iliac artery
- E) External iliac artery

14- Folic acid supplement reduces the risk of

- A) Anencephaly
- B) Preeclampsia
- C) Down syndrome
- D) Placenta previa
- E) Congenital heart disease

15- Preterm premature rupture of the membranes is most strictly defined as spontaneous rupture at any time prior to which of the following

- A) Fetal viability
- B) Second stage of labor
- C) The 32nd week of gestation
- D) Onset of labor
- E) The 37th week of gestation

16- First trimester abortion is mostly due to

- A) Cervical insufficiency
- B) Aneuploidy
- C) Toxoplasma
- D) Trauma
- E) Uterine septum

17- Misoprostol has been found to be effective in all of the following except

- A) Missed abortion
- B) Induction of labour
- C) Menorrhagia
- D) Prevention of postpartum haemorrhage
- E) Management of atonic postpartum haemorrhage

18- A woman complains of sudden onset of watery vaginal discharge at a period of amenorrhea 32 weeks. What is the first step in management

- A) Commence broad spectrum antibiotics
- B) Commence corticosteroids
- C) Perform sterile speculum examination
- D) Perform ultrasound scan to assess Amniotic fluid volume
- E) Commence tocolysis

19- Variable deceleration denotes

- A) Head compression
- B) Cord compression
- C) Placental insufficiency
- D) Fetal distress
- E) Head descent

20- Ventouse in second stage of labor is contraindicated in

- A) Persistent occipito posterior position
- B) Heart disease
- C) Uterine inertia
- D) Preterm labor
- E) Fetal distress

III) Family Medicine Questions

Q1. Enumerate 3 red flags symptoms during Antenatal care period (3 Marks, each for mark)

Q2. Enumerate four health education messages to be given to twenty-two years old P1+0 female seeking Antenatal care (2marks, each 0.5 mark)

Q3. List three common emotional and psychological postpartum problems

Q4. List for educational message given to a lactating mother to prevent breast problems (2 mark, each 0.5 mark)

Q5. Mariam, a 32 years old, first time mother, experiencing postpartum depression. She had mood swings, anxiety and difficulty bonding with her baby after giving birth. Her doctor prescribed antidepressant therapy. She wants to start contraception but was concerned about how it might affect her mental and psychological health. She wants long-term reliable contraceptive method.

A- What is the most suitable contraceptive method for Mariam? (1 mark)

B- Enumerate four contraindications for using this method (4 marks, 1 for each)

خَتَامًا لَا يَجِبُ أَنْ نَنْسِيَ، فَمَنْ نَسِيَ قَضِيَّتَهُ قَدْ بَاعَ، وَمَنْ بَاعَ قَدْ خَانَ، وَمَنْ
خَانَ ضَيَّعَ أَمَّتَهُ...

اللَّهُمَّ كُنْ لِإِخْوَانِنَا فِي فَلَسْطِينَ عَوْنًا وَنَصِيرًا، وَمُؤَيِّدًا وَظَهِيرًا، اللَّهُمَّ ثَبِّتْ
أَقْدَامَهُمْ، وَسَدِّدْ رَفِيقَهُمْ، وَارْزُقْهُمْ عَلَى قُلُوبِهِمْ، وَاحْفَظْهُمْ بِحِفْظِكَ، وَاكْفِهِمْ
شَرَّ أَعْدَائِكَ.

اللَّهُمَّ أَفْرِغْ عَلَيْهِمْ صَبْرًا، وَانْصُرْهُمْ نَصْرًا عَزِيزًا، وَارْحَمْ شُهَدَاءَهُمْ، وَاشْفِ
جَرَاحَهُمْ، وَفُكِّ أَسْرَاهُمْ، وَرُدِّهِمْ إِلَى أَهْلِيهِمْ غَانِمِينَ مُنْتَصِرِينَ.

اللَّهُمَّ أَرِنَا فِي عَدُوِّكَ وَعَدُوِّهِمْ عَجَائِبَ قُدْرَتِكَ، وَخُذْهُمْ أَخْذَ عَزِيزٍ مُقْتَدِرٍ.

